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Nicholas Scarr <nscarr@quantlogic.com>

Re: Qlaim 1816 N Scarr - Fw: Our Ref: 90964727158, Insurer Ref: LBGSBP0073077, Policyholder: Mrs Louisa Scarr, Mr Nicholas Scarr SO24 9BE @909647271:58 #Adjuster#Correspondence#

Nicholas Scarr <nscarr@quantlogic.com> 17 March 2026 at 16:52
To: Simon Fenwick <SFenwick@qlaims.com>
Cc: Claims <Claims@qlaims.com>, Sarah Romain <sarah.romain@qlaims.com>, Andy Lyne <Andy.Lyne@qlaims.com>, Louisa Scarr <scarrlouisa@gmail.com>

Hi Simon,

Thank you for the update.

I have attached the correspondence with Darren Southwell. As you will see from point 4, he confirms that there is no economically viable way to certify these materials as safe.

I have also attached a research report I prepared in early February for your information. It provides a clear overview of the position. This is one of a number of reports I have produced, and every one of them has reached the same conclusion: these materials cannot be considered safe.

The point that must now be pressed firmly upon insurers is this: **the issue is one of proportionality to risk.** ***The cost of replacing clothing is negligible when set against the potential consequences of retaining contaminated items, including life-threatening fungal infection, sepsis, or even fatality. The potential liability exposure in such circumstances would be far greater than the cost of replacement.***

In those circumstances, it is simply not tenable for insurers to continue resisting classification of these contents as BER. There is no safe, reliable, or economically viable means of certifying them fit for use, and the position on risk is now abundantly clear.

I will be writing to Danny Chambers MP later this week to raise a number of serious complaints. The only remaining question is whether the contents issue will form part of that correspondence, or whether insurers will now take the sensible course and agree to classify the affected items as BER.

To be absolutely clear, there is no scenario in which we will accept the return of these items in light of what is now known about the risks they present.

Kind regards,
Nicholas

On Tue, 17 Mar 2026 at 11:57, Simon Fenwick <SFenwick@qlaims.com> wrote:

> Hi Nicholas
>
> Thank you for you email and the update on your positioning which reflects discussion we had last week. I'm aware Andy has been chasing Garrett for a response on your paper and as yet not received a response in this regard as is awaiting details from underwriters and also still seems to be awaiting details from Bio Response.
>
> I have this morning escalated with Lloyds to see if we can gain further traction and ultimately understand underwriters decision, as soon as I have any update I will let you know (unless you or Andy hear first that is!).
>
> Kind regards
>
> Simon

Simon Fenwick

Chief Operations Officer

Qlaims Limited

+44 (0) 7967 223 910

From: Nicholas Scarr <nscarr@quantlogic.com>
Sent: 16 March 2026 17:02
To: Simon Fenwick <SFenwick@qlaims.com>; Claims <Claims@qlaims.com>
Cc: Sarah Romain <sarah.romain@qlaims.com>; Andy Lyne <Andy.Lyne@qlaims.com>; Claims <Claims@qlaims.com>; Louisa Scarr <scarrlouisa@gmail.com>
Subject: Fwd: Qlaim 1816 N Scarr - Fw: Our Ref: 90964727158, Insurer Ref: LBGSBP0073077, Policyholder: Mrs Louisa Scarr, Mr Nicholas Scarr SO24 9BE @909647271:58 #Adjuster#Correspondence#

Dear Simon,

Please see below the email we sent to insurers seven months ago, which clearly set out our position.

I also provided my contents paper on 11 February. To date, we have received no substantive response.

The position is now clear. All of our research and the expert input available to us indicate that the contents identified in my contents paper cannot be evidenced as safe. In those circumstances, we seek urgent written confirmation that they will be treated as beyond economic repair.

Darren Southwell of Bio Response told me that his preferred course would be to dispose of all of the contents referred to in my paper, as there is no economically viable way of evidencing that they are safe. He also made clear that he would not accept those contents himself unless he could be satisfied that they were safe. Given Louisa's medical vulnerability, that is plainly the only responsible position.

If these items cannot be evidenced as safe in any economically realistic manner, they should be written off. Insurers cannot properly maintain a position whereby they are unable to demonstrate that the contents are safe, accept that meaningful verification would be prohibitively expensive, and yet continue to press for retention.

In Louisa's case, that is particularly serious. Her vulnerability means that any expectation that we should accept uncertified and unevidenced contents places her in a position of foreseeable serious harm. That is not a risk we can or will accept.

We therefore ask for urgent confirmation that the contents identified in my contents paper will now be classified as beyond economic repair and dealt with accordingly.

Yours sincerely,

Nicholas Scarr

Nicholas Scarr

---------- Forwarded message ---------
From: **Nicholas Scarr** <nscarr@quantlogic.com>
Date: Tue, 19 Aug 2025 at 14:56
Subject: Qlaim 1816 N Scarr - Fw: Our Ref: 90964727158, Insurer Ref: LBGSBP0073077, Policyholder: Mrs Louisa Scarr, Mr Nicholas Scarr SO24 9BE @909647271:58 #Adjuster#Correspondence#
To: Andy Lyne <Andy.Lyne@qlaims.com>

Cc: Sarah Romain <sarah.romain@qlaims.com>, Louisa Scarr <scarrlouisa@gmail.com>, Claims <Claims@qlaims.com>

Dear Andy,

This correspondence sets out our unequivocal position regarding the health and environmental risks posed by the sewage contamination incident at our property, and the consequent requirement for the permanent removal of contaminated materials.

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1. Medical Vulnerability of Mrs Louisa Scarr

Mrs Louisa Scarr's medical history places her in one of the highest clinical risk categories for microbial exposure. Her record includes:

• Clostridioides difficile (C. difficile) – a spore-forming, toxin-producing bacterium capable of persisting in the environment and resistant to standard disinfectants;

• Klebsiella pneumoniae (NDM strain) – a carbapenem-resistant "superbug" designated a critical priority by the World Health Organization due to limited treatment options and high mortality risk;

• Sepsis – an episode so severe in 2013 that critical care intervention and the administration of last rites were required.

These infections severely compromised Mrs Scarr's gastrointestinal and systemic immune defences. The risk of recurrence remains significant, especially with environmental exposure to spores and resistant organisms.

The attached medical letter from Dr. Van Gant underscores this vulnerability. He confirms that the only safe standard for Mrs Scarr's return to the property is one equivalent to "hospital clean". Nothing short of this would protect her from foreseeable microbial harm.

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2. Timeline and Environmental Conditions

• Initial Exposure Period: Raw sewage remained on the ground floor for approximately 20 weeks. Beyond an initial superficial clean, the contamination was untreated and pooled under floorboards until Bio Response began professional drying and sanitation.

• Rat Infestation: The prolonged presence of sewage and organic waste caused a severe rat infestation, described by our pest control specialist (an industry veteran) as one of the worst he has encountered.

• Bio Response Findings: Bio Response advised unequivocally that the kitchen units were compromised and required full removal. They also identified widespread microbial and moisture damage throughout both floors, including contamination of household furnishings and electronics such as the Everhot cooker and t televisions.

• Strip-Out Observations: When strip-out began more than three months after the incident, there was still visibl dampness, sewage odours, and residual pooling beneath surface structures.

• Upstairs Contamination Indicators: Moisture cracking, persistent odours, and dampness on the first floor suggest airborne microbial migration and humidity-driven contamination.

• Sanitisation Limitations: Despite disinfectant application, ATP testing at the site of the former Everhot kitchen unit recorded a reading of 27+, well above safe thresholds and confirming active biological contamination.

• Drying and Scrubbing Measures: While industrial drying machines and air scrubbers are deployed, they cannot reverse the prolonged contamination or render porous materials safe.

Airborne Pathogen Risk: Industry authorities are explicit on this point:

• The CDC warns that sewage releases generate bioaerosols capable of travelling well beyond the immediate contamination site.

• The EPA notes that sewage-related pathogens may remain suspended in indoor air, creating inhalation risks long after the initial incident.

• The IICRC S500 and S520 standards confirm that porous and semi-porous building materials impacted by sewage cannot reliably be sanitised to safe levels.

Taken together, these findings explain why the property has remained uninhabitable and microbially unstable for nearly five months, with the risk of airborne dissemination across both floors.

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3. Rejection of Sampling as a Viable Approach

At the outset, Bio Response advised clearly that effective decontamination of porous materials exposed to sewage could not be guaranteed. Their advice was that:

• Pathogens—particularly spores and resistant bacteria—would remain embedded in woodwork, insulation, kitchen units, and soft furnishings;

• Surface swab testing would be neither scientifically conclusive nor commercially viable;

• Two small swabs per item (the apparent industry norm) would be wholly inadequate given the scale of contamination;

• Even comprehensive microbial testing would exceed the value of affected items and still fail to offer 100% safety assurance.

If any later suggestion from Bio Response to insurers contradicts this, it would stand at odds with the clear in-person guidance we received during assessment. Their position at the time was unambiguous: kitchen units, sewage-exposed furnishings, and electronics were not recoverable and should be removed.

———

4. Justified Material Removal

Following both expert advice and recognised international remediation protocols (CDC, EPA, IICRC, UK HSE), the following items must be removed:

• All kitchen units, due to sewage contact, structural compromise, and microbial absorption;

• All soft furnishings, including mattresses, carpets, upholstered furniture, and all fabric-based materials, both on the ground and first floors. Their porous nature and exposure make them impossible to render safe;

• All affected electronics, where exposure to moisture and airborne contaminants renders them unsafe.

———

5. Summary Position

Our position is grounded in both medical necessity and environmental science. We reaffirm that:

• Porous, sewage-exposed materials cannot be rendered safe by sanitisation;

• Airborne pathogen risks extend contamination beyond visible areas;

• For a healthy household, this exposure would still be unacceptable—yet for Mrs Scarr, it is clinically dangerous;

• As confirmed by Dr. Van Gant, nothing less than hospital-grade cleanliness will make the property safe for reoccupation;

• Retaining contaminated materials, even after disinfection or speculative testing, presents a foreseeable and unacceptable risk to health.

In view of the very long delays already endured, we respectfully request that authorization for the removal of the contaminated materials be sanctioned within the next seven days.

Accordingly, the removal of the specified items is not optional but medically and environmentally essential. We ask that this position, together with the attached letter from Dr. Van Gant, be formally recorded in the claims file.

We do not seek confrontation, but our stance is evidence-based, consistent with clinical advice, and not open to dilution.

Yours sincerely,

Nicholas

Nicholas Scarr

Sent from my iPad

2 attachments



Bio Response 14.11 Darren Southwell.jpg
3050K

Forensic Sewage Contamination Report Prompt (2).pdf
1837K